

ROI models can feel like black boxes—numbers that sound good but don't explain themselves. This assessment is different. Every value traces back to your inputs, industry benchmarks, and assumptions you can inspect. We're not selling you on a number. We're giving you a model you can stress-test, adjust, and defend internally.

You selected **3 value drivers**: Patient Throughput (LWBS Reduction), Level-of-Service Accuracy, Documentation-Related Denials. Each section walks through the logic step by step—what we're measuring, why it matters, and exactly how we calculated it.

+\$264K
NET ANNUAL GAIN
\$331K value - \$68K cost

4.9x
RETURN ON INVESTMENT
Every \$1 returns \$4.91

\$11K
PER ED PROVIDER
Net annual benefit each

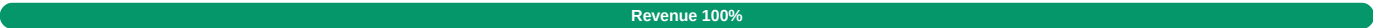
1,083
HOURS RETURNED
Documentation time saved

WHERE THE VALUE COMES FROM

LABOR & EFFICIENCY
0% of total value
\$0
No labor drivers selected

REVENUE & QUALITY
100% of total value
\$331K

Patient Throughput (LWBS Reduction)	\$123K
Level-of-Service Accuracy	\$107K
Documentation-Related Denials	\$101K



INVESTMENT DETAILS

YOUR CONFIGURATION

Setting	Emergency Department
ED providers	25
Price	\$225/ED provider/month
Annual Investment	\$68K

MULTI-YEAR PROJECTION

	Year 1	Year 2	Year 3	3-Yr Total
Value	\$331K	\$364K	\$401K	\$1.10M
Cost	\$68K	\$68K	\$68K	\$203K
Net	\$264K	\$297K	\$333K	\$894K

Assumes 10% annual value growth with increased adoption

At **\$11K per ED provider** in net annual value, scaling from 25 to 75 ED providers would increase annual benefit from \$264K to approximately \$791K. The methodology section explains how these projections work—and where your situation might differ.

Patient Throughput (LWBS Reduction)

\$123K

THE THEORY

When patients leave without being seen (LWBS), the ED loses revenue and—more importantly—fails its core mission. LWBS happens for many reasons: long wait times, staffing constraints, perceived non-urgency. But documentation delays play a role too.

When physicians can complete documentation faster, disposition happens sooner. Patients move through the department more quickly. Wait times compress. And fewer patients give up and leave.

We model this conservatively: only 40% of LWBS is attributable to documentation-driven bottlenecks, and Abridge can address about half of that. The rest is driven by factors outside documentation—true capacity constraints, triage decisions, patient behavior.

YOUR CALCULATION

STEP 1: LWBS PATIENTS

How many patients leave without being seen?

26,000

visits x

3.5%

 =

910 LWBS

Industry LWBS rates vary from 2-5%. High-volume urban EDs often run higher.

STEP 2: DOCUMENTATION-ATTRIBUTABLE

How much LWBS is documentation-related?

910

LWBS x

40%

 =

364 doc-related

Not all LWBS is documentation-related. We estimate 40% is tied to throughput delays that documentation impacts.

STEP 3: ABRIDGE IMPACT

How many can Abridge help recover?

364

doc-related x

50%

 =

182 recovered

50% reduction is achievable when documentation becomes synchronous with care. Some organizations see more.

STEP 4: REVENUE

What's the financial impact?

182

recovered x

\$350

 =

\$123K

Average ED revenue per visit. Adjust based on your payer mix and acuity.

BENCHMARK: ED REVENUE PER VISIT

Low Acuity (ESI 4-5)	\$150 - \$250
Medium Acuity (ESI 3)	\$300 - \$450
High Acuity (ESI 1-2)	\$600 - \$1,200+
Blended Average	~\$350

Your input reflects a blended average across your acuity mix.

Annual Patient Throughput (LWBS Reduction) Value

182 recovered x \$350/visit

\$123K

WHAT THIS MEANS

At \$123K, throughput improvements contribute to your total value. This represents approximately 350 patients who would have left but now complete their visit. Beyond the revenue, there's a quality dimension: patients who leave without being seen may delay necessary care or present later with worse outcomes. Reducing LWBS is both a financial and clinical win.

Level-of-Service Accuracy

\$107K

THE THEORY

ED visits are often complex—multiple problems, rapid assessment, time-sensitive decisions. But documentation doesn't always reflect that complexity. Under time pressure, notes get truncated. MDM is summarized rather than detailed. The result: visits that should code at 99285 get billed as 99284.

Abridge captures the clinical narrative as it happens. When the note includes the full reasoning—why you ordered that CT, what differential you were considering, how the patient's presentation evolved—coding can be more accurate.

We model a 5% wRVU improvement, consistent with what we see across ED deployments. Your results depend on current documentation quality and specialty complexity.

YOUR CALCULATION

STEP 1: BASELINE WRVUS

What's your current wRVU generation?

26,000 visits x 2.5 = 65,000 wRVUs

STEP 2: WRVU IMPROVEMENT

How much lift does better documentation create?

65,000 wRVUs x 5% = 3,250 wRVU gain

5% improvement assumes meaningful under-documentation today. If your notes are already comprehensive, improvement may be less.

STEP 3: REVENUE

What's the dollar value?

3,250 wRVUs x \$33 = \$107K

Medicare conversion factor. Commercial rates run \$45-65.

BENCHMARK: ED WRVU PER ENCOUNTER

Low Acuity (ESI 4-5)	1.2 - 1.6
Medium Acuity (ESI 3)	2.0 - 2.5
High Acuity (ESI 1-2)	3.0 - 5.0+
Blended Average	~2.2

ED wRVUs vary by acuity mix. 2.0-2.5 per encounter is typical.

Annual Level-of-Service Accuracy Value

3,250 wRVU gain x \$33

\$107K

WHAT THIS MEANS

At \$107K, level-of-service accuracy contributes to your total value. This represents getting appropriate credit for the complexity of care you're already delivering. The improvement comes from capturing the full clinical narrative—not upcoding, just accurate documentation of what you're already doing.

Documentation-Related Denials

\$101K

THE THEORY

ED claims face unique denial challenges. The acuity that justified the visit isn't always clear in the note. The medical necessity for a procedure gets questioned. The MDM that seemed obvious in the moment doesn't translate to paper.

These denials are often recoverable—you appeal, you win. But the rework is expensive, and some claims get written off because the cost of appeal exceeds the value.

Abridge captures the clinical reasoning in real time. When that reasoning is in the note, denials are less likely—and when they happen, appeals are more defensible.

YOUR CALCULATION

STEP 1: TOTAL DENIALS

How many claims are denied today?

26,000

 visits x

10%

 =

2,600 denials

ED denial rates often run 8-12%—higher than outpatient due to complexity.

STEP 2: DOCUMENTATION-RELATED

How many are caused by documentation gaps?

2,600

 denials x

40%

 =

1,040 doc denials

35% of ED denials are documentation-related: missing MDM, unclear necessity, incomplete notes.

STEP 3: WRITTEN OFF

How many are lost without appeal?

1,040

 doc denials x

30%

 =

312 written off

These are claims abandoned without appeal.

STEP 4: ABRIDGE RECOVERY

How many can Abridge save?

312

 written off x

50%

 =

156 recovered

75% recovery when the clinical reasoning exists but wasn't documented.

STEP 5: VALUE

What's the dollar impact?

156

 claims x

\$650

 =

\$101K

ED claims average higher than outpatient due to acuity.

BENCHMARK: AVERAGE ED CLAIM VALUE

Low Complexity	\$175 - \$250
Medium Complexity	\$300 - \$450
High Complexity	\$500 - \$800+
Blended Average	~\$350

Annual Documentation-Related Denials Value

156 claims x \$650

\$101K

WHAT THIS MEANS

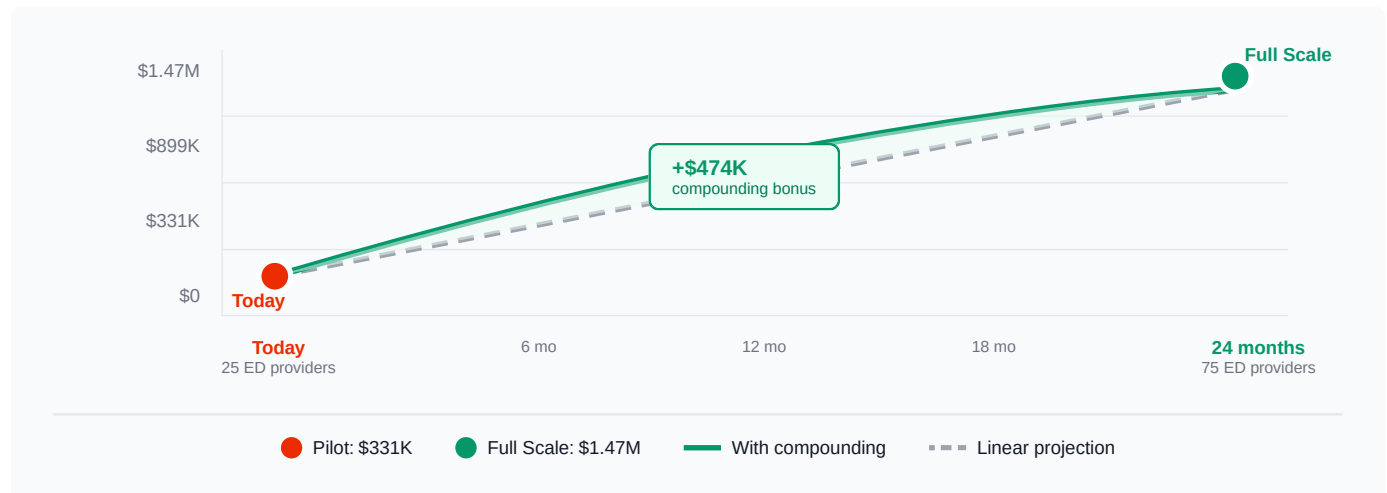
At \$101K, denial prevention contributes to your total value. This represents approximately 290 claims that would otherwise be written off. Abridge captures the clinical reasoning that either prevents denials or makes them winnable on appeal.

Your Journey with Abridge

Start with a pilot. Prove the value. Scale across your organization.

The ROI you see today reflects a pilot deployment of 25 ED providers at 65% utilization. As you scale, value compounds—not just linearly with ED provider count, but exponentially as utilization matures, workflows optimize, and network effects emerge.

At full scale (75 ED providers, 80% utilization), annual value reaches **\$1.47M**—a **4.4x increase** from pilot.



MODEL YOUR SCENARIO

YOUR STARTING POINT

- 25 ED providers
- 40,000 patient visits
- 65% utilization
- \$331K /year

YOUR FULL SCALE POTENTIAL

- 75 ED providers
- 80% utilization
- Scaling: 24 months
- \$1.47M /year

WHY VALUE COMPOUNDS

4.4x

Value doesn't just scale linearly with ED provider count. As utilization improves (65% to 80%), workflows optimize, and network effects emerge, each ED provider generates more value at maturity than at pilot.

DOWNSTREAM VALUE

The ED admission note is just the beginning

When an ED physician decides to admit a patient, their documentation becomes the foundation for inpatient revenue. The conditions they capture, the medical necessity they establish, and the clinical reasoning they document all determine what happens downstream.

Better ED documentation directly impacts:

DRG & CMI Capture

CCs and MCCs documented in ED carry forward to inpatient coding. What's captured here determines your case mix.

Medical Necessity

The admission decision is documented in ED. This is your first line of defense against status denials and downgrades.

CDI Efficiency

When the ED note is complete, CDI teams spend less time querying physicians and more time on complex cases.

Denial Prevention

Payer audits start with the admission note. Complete documentation from day one means stronger appeals.

These benefits are quantified in the **Inpatient Setting**.
If your organization admits patients from the ED, the value compounds when both settings use Abridge.

Methodology & Assumptions

YOUR INPUTS

25 ED providers
40,000 annual patient visits
65% utilization rate
2.5 min saved per encounter
\$225/ED provider/month

ED-SPECIFIC BENCHMARKS

wRVU per encounter: 2.0-2.5
ED physician replacement: \$500K+
Average claim value: \$350
ED turnover rate: 8-12%
ED denial rate: 8-12%

CALCULATION PRINCIPLES

Conservative estimates throughout
Medicare rates (not commercial)
Transparent, auditable logic
No compounding in Year 1
10% annual growth for Y2-Y3

All benchmarks based on aggregate data from 200+ health system partners. Individual results vary based on patient acuity mix, payer mix, operational factors, and implementation quality. We use conservative assumptions throughout—actual value may be higher.

"Emergency departments face unique documentation pressures—high volume, high acuity, time pressure. The difference between a 2x ROI and a 5x ROI usually isn't the technology. It's utilization, change management, and knowing which drivers matter most for your ED's situation."